

Pemetrexed monotherapy

Lung · NCCP 00222 · version 5

Your medicines

Pemetrexed

Your treatment schedule

Every 21 days · See page 2 for the treatment phases.

A picture of side effects

Tell your team about new or worsening symptoms. The same symptom can have more than one cause. Numbers connect each symptom box to its location.

Pemetrexed · chemotherapy

Blood / kidney / mouth / skin effects

1 Blood / infection

Pemetrexed

Low blood counts can increase infection, bruising or breathlessness. Fever or sudden illness needs prompt oncology advice.

3 Mouth / bowel

Pemetrexed

Mouth soreness, diarrhoea or vomiting can occur. Contact the team if eating, drinking or hydration are affected.

5 Breathing

Pemetrexed / treatment context

New breathlessness or cough needs assessment because infection, cancer, clot or treatment toxicity can overlap.

2 Kidneys

Pemetrexed

Kidney function affects treatment suitability. Report reduced urine, swelling or significant dehydration.

4 Skin

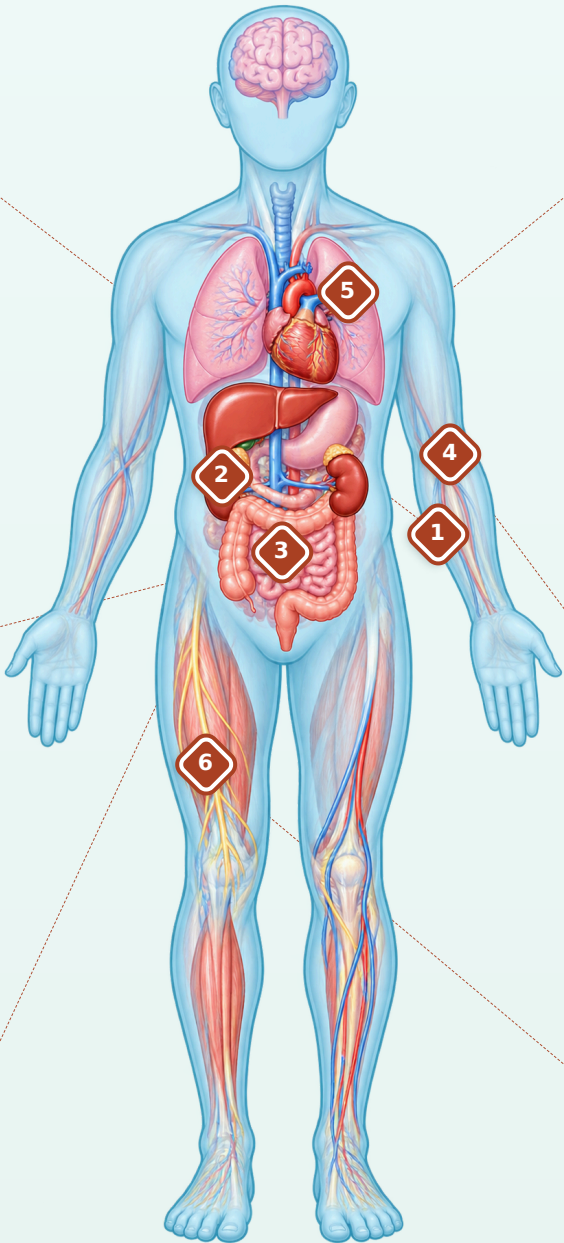
Pemetrexed

Rash can occur. Take the prescribed steroid prophylaxis and report a severe or spreading rash.

6 Energy / daily function

Pemetrexed

Report fatigue or weakness that is new, severe or interfering with normal activities.



This map shows selected effects to discuss; it does not show their likelihood or every possible side effect. Blood-count and whole-body effects are marked schematically.

Selected symptom patterns for discussion. Tell your oncology team about any new or worsening symptom. See page 2 for medicine-specific effects, monitoring and the portal QR.

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TREATMENT JOURNEY

Day 1 Pemetrexed 500 mg/m ² by IV infusion.	Every 21 days Treatment repeats every 21 days if review and blood tests allow.	During treatment Take folic acid as prescribed; vitamin B12 support and steroid rash prophylaxis are part of pemetrexed care.	Duration For maintenance/second-line use, treatment may continue until disease progression or unacceptable toxicity, according to the treatment plan.
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IMPORTANT MEDICINE-SPECIFIC EFFECTS

Pemetrexed

- Low blood counts can increase infection, anaemia or bruising risk.
- Mouth/gut inflammation and skin rash can occur.
- Kidney function can deteriorate and is monitored.

Other / shared treatment effects: The anatomy page highlights important symptom patterns for this exact regimen, but it cannot show every possible effect. Fatigue, nausea, appetite change, mouth/bowel symptoms or blood-count changes may occur depending on the medicines used. Report new or worsening symptoms even if they are not listed here.

Monitoring / medicines • FBC, kidney and liver profile are checked before each cycle. • Folic acid must be taken as prescribed during therapy and after the final pemetrexed dose. • Vitamin B12 is given according to the regimen schedule.	Bring / track • A corticosteroid is used around pemetrexed to reduce skin reactions. • Bring a list of current medicines and report new symptoms before the next dose.
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WHEN TO CALL - DO NOT WAIT FOR THE NEXT APPOINTMENT

Contact oncology urgently for fever or sudden illness, new significant breathlessness, uncontrolled vomiting or diarrhoea, severe mouth soreness preventing fluids, marked reduction in urine, significant bleeding or a rapidly worsening rash. For a medical emergency call 112/999.

Consent discussion support

Use the approved consent process to document treatment aim, expected benefit, material risks, alternatives, questions and the patient's decision.

Scan to open the regimen-specific patient portal

Regimen information only; this QR contains no patient details.

<https://sactcheck.com/patient/00222/>

Official NCCP 00222 v5 · SACTCheck v0.81.2 · patient information prototype for clinical review.

